CLINICAL TRIAL FAILURES · MISSED ENDPOINTS · ONCOLOGY PHASE 2,3 · STARTS 2015–2024

LAG-3 + PD-(L)1: 8 of 8 completed trials missed their primary endpoint

8 completed trials across 4 sponsors and 11 molecules ran to the end and missed their primary endpoint on the sponsor's own posted result; 0 met it. Only 1 of 65 closed trials were stopped early for efficacy or safety, which is why this class is read through its completed trials rather than a stop rate. Most completed trials post nothing readable, so the count is a floor.

8 of 8
completed trials with a readable primary result missed it · 0 met it

4 / 11
sponsors / molecules behind the misses

1.5%
stopped early: 1 of 65 closed trials (95% CI 0.3%–8.2%)

52.4%
of 124 trials have closed · 59 still open or unresolved

8 of 8 **Completed and missed the primary endpoint.** Of 49 completed trials in this segment, 8 posted a primary result on ClinicalTrials.gov that can be read — the sponsor's own comparison held to the sponsor's own threshold, or its statement that the endpoint was missed: 8 missed and 0 met. Counted separately and never in the rate above; a floor, not a rate. 6 of the 38 completed trials that finished more than a year ago have posted no results at all — a gap in what can be known, not a sign of failure.

The completed trials that missed

Trial	Ph	Sponsor	Experimental drugs	Started	What the posted result says
NCT05775289	2	Roche	TOBEMSTOMIG	2023-03	Progression-Free Survival (PFS) — p 0.9705, no threshold stated; 0.2 or above, so not significant under any conventional threshold. Hazard Ratio (HR) 0.99 (95% CI 0.63 to 1.56). Method: Log Rank.
NCT05287113	2	Incyte	TUPARSTOBART, VERZISTOBART	2022-11	Progeression-free Survival (PFS) — p 0.5856, no threshold stated; 0.2 or above, so not significant under any conventional threshold. Hazard Ratio (HR) 1.05 (95% CI 0.66 to 1.67). Method: Log Rank.
NCT05337137	2	Bristol Myers Squibb	RELATLIMAB	2022-05	Objective Response Rate (ORR) Assessed by BICR - All Comer Population — Mean Difference (Final Values) -6.5 (90% CI -23.8 to 10.7), no p-value posted and no threshold stated: the interval is consistent with no effect under any conventional threshold, so not significant.
NCT05116202	2	Roche	TOBEMSTOMIG, ATEZOLIZUMAB, TIRAGOLUMAB	2022-02	Pathologic Response Rate (pRR) for Cohort 1 as Determined by Independent Pathologic Review — Difference in pRR 2.73 (95% CI -22.25 to 27.7), no p-value posted and no threshold stated: the interval is consistent with no effect under any conventional threshold, so not significant.
NCT05064059	3	Merck & Co.	FAVEZELIMAB, PEMBROLIZUMAB	2021-11	Overall Survival (OS) — one-sided p 0.4183, no threshold stated; 0.2 or above, so not significant at any one-sided threshold in use. Hazard Ratio (HR) 0.98 (95% CI 0.8 to 1.2). Method: Log Rank.
NCT04785820	2	Roche	LOMVASTOMIG, TOBEMSTOMIG	2021-06	Overall Survival (OS) — p 0.7689, no threshold stated; the sponsor posted a 80% interval, i.e. a design tested at two-sided 0.2: not significant. Hazard Ratio (HR) 1.05 (80% CI 0.84 to 1.33). Method: Log Rank.

The 6 most recently started of 8 completed trials that missed. The diligence report lists every one, with the numbers or the sentence each verdict was read from.

How this compares

LAG-3 + PD-(L)1



1.5%

1/65 closed

PD-(L)1 combinations without LAG-3



5.4%

116/2153 closed

All oncology Phase 2,3



4.5%

356/7956 closed

What is in this class: 130 distinct experimental drugs across 124 trials (antibody 122, small molecule 36, unknown 9, protein 2). Most tested: RELATLIMAB (48), NIVOLUMAB (43), PEMBROLIZUMAB (18), IPILIMUMAB (16), FAVEZELIMAB (13). A class groups drugs by what they act on; check that the grouping is one you would make before reading the rate as a property of the mechanism.

How much does this rest on one decision? The one stop came from 1 sponsor-asset programme across 1 sponsor; the largest (MacroGenics) contributed 1; removing that programme's trials from both sides leaves 0.0%. Screened alongside 85 other segments, the rate is within what screening that many classes could produce (q=1); the molecules and trials that failed here are the record either way. A further 4 closed trials here were terminated with no cause recorded in the registry; if every one of them were biological the rate would be 7.7%. Read it as a screen worth checking against the underlying trials.

Bars show the share of closed trials stopped for biological reasons. Confidence intervals overlap where sample sizes are small — read the counts, not just the bars.

By start cohort

Trials started	Stops	Closed	Rate	95% CI
2015–2017	0	8	0.0%	0.0%–32.4%
2018–2020	0	28	0.0%	0.0%–12.1%
2021–2024	1	29	3.4%	0.6%–17.2%

Where the stops sit

Safety: 1.
Sponsors with most stops: MacroGenics (1).

The stopped trials

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT04634825	2	MacroGenics MGNX	ENOBLITUZUMAB, RETIFANLIMAB, TEBOTELIMAB	2022-07	Safety	Based on internal review of safety data

Method
Denominator: ClinicalTrials.gov interventional Phase 2,3 oncology trials started 2015–2024 that have closed (completed or terminated). Numerator: terminated trials whose registry stop reason is classified as biological (efficacy, safety or benefit–risk) by Classification V2 — held-out precision 95.5%, recall 95.3% (n=600). Drugs are linked to ChEMBL and the NCI Thesaurus; 76% of industry oncology trials in this window carry a resolved drug target, and a trial without one cannot enter a mechanism class. Intervals are Wilson 95%.

Limits
Not a failure rate: completed trials that missed their endpoints are not in the rate. They are counted separately where the sponsor posted a primary result that can be read — most did not, so that count is a floor — and programmes discontinued after a completed trial do not appear. Stop reasons are sponsor-reported. This is a closed-trial proportion, not a time-to-event analysis: only 52.4% of trials in this segment have closed, and a trial that stops early enters the denominator sooner than one that runs to completion, which can inflate the rate in immature segments. Recent cohorts have fewer closed trials, so their rates are less stable. Research signals, not clinical or investment advice.

The whole cohort, trial by trial. The diligence report names every trial behind these figures — the ones still running and the ones that completed and missed their primary endpoint included — and places your molecule against each one that failed. Free during the beta: enter your work email at **clinicaltrialfailures.com/asset-check** and it opens straight away.

Sources: ClinicalTrials.gov (NLM); ChEMBL (EMBL-EBI, CC BY-SA 3.0); NCI Thesaurus (NCI); RxNorm/RxClass (NLM); SEC EDGAR. Classification, linkage and rates by Clinical Trial Failures. Rebuilt weekly; this brief covers trials started 2015–2024.